

Research Briefing

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The regulation of non-surgical cosmetic procedures in England



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Summary

This briefing provides an overview of the regulation of non-surgical cosmetic procedures in England.

There is no specific definition or comprehensive list of these procedures. However, they are [widely understood to be](#) non-invasive treatments that are delivered without requiring a cut to the skin, or using surgical instruments. Examples of non-surgical cosmetic procedures include botulinum toxin ('Botox') injections, dermal fillers, chemical peels and laser hair removal.

There are no specific controls on who can offer non-surgical cosmetic procedures, although cosmetic dentistry is clearly restricted to registered dental professionals.

Concerns about the safety of non-surgical cosmetic procedures

There has been long-standing and established concern about the risk of harm members of the public face when undertaking cosmetic procedures.

A number of investigations and reports have been carried out into regulation in the cosmetic procedure sector, including:

- The [Review of the Regulation of Cosmetic Interventions](#) (the Keogh Review, 2013), commissioned by the then Secretary of State for Health Andrew Lansley
- [Health Education England's review of qualification requirements for non-surgical cosmetic procedures](#) (2013-2015), commissioned by the then Department of Health
- The Nuffield Council on Bioethics report, [Cosmetic procedures: ethical issues](#) (2017)
- The Health and Social Care Committee report, [The Impact of body image on mental and physical health](#) (2020)
- The All-Party Parliamentary Group on Beauty, Aesthetics and Wellbeing, Concluding report: [Inquiry into advanced aesthetic non-surgical cosmetic treatments](#) (PDF), 2021

All recommended that the government take steps to introduce further regulation in the sector, and particularly around introducing minimum education and training requirements, as well as centralised registration, for practitioners.

In recent years, there have been many [reports of people having suffered harm caused by ‘botched’ procedures](#), and these have been linked to concerns about lack of regulation.

Regulation of practitioners and procedures

There is no specific requirement for practitioners to undertake education or training before delivering non-surgical cosmetic procedures.

Practitioners can choose to adhere to voluntary regulatory mechanisms. For example, they can join voluntary registers maintained by the [Joint Council for Cosmetic Practitioners \(JCCP\)](#) and [Save Face](#), which require registrants to meet training, education, clinical and professional conduct standards. Practitioners can also choose to meet practitioner [education and training standards recommended by Health Education England](#).

If practitioners are also regulated healthcare professionals, they will generally be required to ensure that their cosmetic practice is in accordance with any clinical guidance or professional standards set by their professional regulators. Regulators can take action against healthcare professionals, even if their cosmetic practice is outside the scope of their normal work.

A new cosmetic licensing scheme for England

Section 180 of the [Health and Care Act 2022](#) enables the UK Government to establish a licensing scheme for non-surgical cosmetic procedures for England and Wales.

Between September and October 2023, the Department of Health and Social Care (DHSC) held a [consultation on the design of the new scheme](#).

The government published its [response to the consultation in August 2025](#), where it set out details of its proposals on how the new scheme would be implemented.

The government proposed placing cosmetic procedures in red, amber or green categories according to the level of risk associated with them:

- Green procedures, considered low risk, could be carried out by any licensed practitioner who meets agreed standards.
- Amber procedures, posing medium risk, could be performed by non-healthcare practitioners only under the oversight of a named regulated healthcare professional, while qualified healthcare professionals could carry them out independently if they meet the required standards.
- Red procedures, judged to carry the highest risk, would be limited to suitably qualified and regulated healthcare professionals and fall under Care Quality Commission (CQC) regulation, meaning they could only be delivered in CQC-registered premises.

The licensing scheme for green and amber procedures would be administered by local authorities.

In its response to the consultation, the government set out an intention to proceed, and to prioritise its work on regulating high-risk procedures (such as those aimed at augmenting the breast, buttocks and genitals with dermal fillers).

The government has also set out an intention to restrict procedures being delivered to under-18s, except with medical oversight.

Wider regulatory considerations for practitioners and businesses

Employers and the self-employed are subject to a range of duties under health and safety law. These broad duties aim to ensure that employers and the self-employed take measures to reduce the risk of harm members of the public are exposed to as a result of the business activity.

Health and safety law is enforced by the Health and Safety Executive, and local authorities, depending on the nature of the business activity and the premises in which they are carried out.

Routes of redress for members of the public if things go wrong

In the event that a member of the public is unsatisfied with a procedure, and particularly if they wish to express concerns about the impact of the procedure on their health or wellbeing, they may consider raising concerns with:

- The practitioner, or their employer.
- A voluntary register, if the practitioner is a registrant.
- A professional health or care regulator, if the practitioner is a registered healthcare professional.
- The [Independent Sector Complaints Adjudication Service \(ISCAS\)](#) if the practitioner has subscribed to the scheme.
- [The Cosmetic Redress Scheme \(CRS\)](#), if the practitioner has subscribed to the scheme. The CRS only considers complaints regarding consumer law, or poor or unsatisfactory service.
- [The Health and Safety Executive](#), or the local authority, if there are concerns that the practitioner or employer has not met their obligations under health and safety law.

None of these routes of redress are a substitute for professional medical advice. Members of the public should always seek advice from a qualified healthcare professional if they are concerned about their health or wellbeing.

1

Background

1 Terminology

In this briefing, we've used the term 'practitioner' to describe individuals that carry out cosmetic procedures on other individuals as part of a working practice.

We refer to those receiving such procedures as 'members of the public', rather than 'patients', because in this context the procedures are not provided for medical purposes.

Non-surgical cosmetic procedures are interventions carried out for cosmetic purposes that do not involve surgery. They are typically minimally invasive and include treatments such as injections, chemical peels, microneedling, and the application of light, heat, or other energy-based devices.

Surgical procedures are generally considered to be invasive operations that involve making a cut or incision to the skin with the use of surgical instruments. While there is no single statutory definition of a surgical procedure, this distinction is widely recognised in medical and regulatory contexts.¹

A growing range of non-surgical cosmetic procedures are available. These include botulinum toxin injections (commonly known as 'Botox'), dermal fillers, laser hair removal, microdermabrasion/ chemical peels and non-surgical liposuction (see table 1).

Alongside this growth, there have been concerns about a lack of regulation in the cosmetic procedure industry.² These have focused on the fact that there are no minimum training requirements for people delivering the procedures (practitioners), and the potential risk of harm this poses to people

¹ See, for example, Save Face, [What are non-surgical cosmetic procedures?](#), accessed 9 September 2025, The Advertising Standards Authority, [Cosmetic interventions: non-surgical procedures](#), 1 July 2025

² See, for example, Chartered Trading Standards Institute, [Urgent regulation of fat injections, fillers and botox needed](#), 18 June 2025

undertaking the procedures.³ There are associated concerns about a lack of redress for members of the public when things go wrong.⁴

The government has taken step towards improving regulation in the cosmetic procedures sector. The [Health and Care Act 2022](#) granted powers for the UK Government to establish a licensing scheme for non-surgical cosmetic procedures for England and Wales.

In August 2025, [the government announced that](#) lower-risk cosmetic treatments, such as botulinum toxin ('Botox'), lip fillers and facial dermal fillers, would be regulated through a new local authority licensing system.⁵ Practitioners would need to meet safety, training and insurance standards in order to legally operate their business. The government also set out plans to restrict high-risk cosmetic procedures for under-18s, unless the procedure is approved by a doctor and carried out by a healthcare professional.

2 Scope of this briefing paper

This briefing provides an overview of how non-surgical cosmetic procedures are regulated in England.

Public health is a devolved matter, and arrangements are within the remit of the devolved executives and legislatures. For this reason, this briefing does not provide information on regulation in Wales, Scotland and Northern Ireland.

This briefing does not provide:

- information on the regulation of cosmetic surgery, as this is subject to a distinct and separate regulatory arrangement,
- discussion on the advertising of cosmetic procedures, or
- information on surgical and non-surgical procedures carried out overseas.

1.1

What kind of cosmetic procedures are being carried out?

There is a wide range of non-surgical procedures available in England. Table 1 provides examples of those commonly referred to.

³ DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

⁴ DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

⁵ DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

The Joint Council for Cosmetic Practitioners (JCCP) has published a range of [information sheets intended for the public](#).⁶ They explain how different cosmetic procedures are carried out, potential benefits, risks and side effects.

The Department of Health and Social Care (DHSC) has published a [glossary of procedures](#) which includes descriptions.⁷

Table 1 Examples of non-surgical cosmetic procedures	
'Botox' injections	"Botox" is the brand name of a product that contains botulinum toxin. This substance temporarily relaxes muscle and is used in the face to reduce the appearance of wrinkles and fine lines. ⁸
Non-surgical body contouring or body sculpting	This term describes a wide range of procedures and methods that are used to augment the size and shape of the body by eliminating fat and tightening skin. Procedures might also be referred to as non-surgical lifts. It can involve the use of cold, heat and lasers. There is a distinction from surgical methods used to achieve these results. Common examples are tummy tucks, surgical lifts and liposuction. ⁹
Non-surgical Brazilian butt lift (BBL)	A non-surgical BBL involves the injection of dermal filler to enhance the size and shape of the buttocks. It may be referred to as a 'liquid BBL'. It is distinct from a surgical BBL, in which the client's own fat is transferred from other areas of the body (e.g. the stomach) and relocated to the buttocks.
Chemical peels	Chemical peels may contain glycolic acid, salicylic acid or lactic acid. These ingredients irritate skin on the face, allowing older skin to peel away. They stimulate the growth of new skin cells to improve the texture and appearance. They can improve the appearance of fine lines, wrinkles, scarring and hyperpigmentation. ¹⁰
Cryotherapy	Cosmetic cryotherapy uses ultra-cooled nitrogen gas to target areas such as the face, scalp and neck to enhance the appearance of the skin. ¹¹
Intense pulsed light (IPL) treatments	IPL uses high-intensity pulses of visible light to improve the appearance of a range of skin problems such as vascular lesions, port wine

⁶ JCCP, [The procedures that are regulated by the JCCP – introduction to non-surgical aesthetic treatments and hair restoration surgery](#), accessed 4 September 2025

⁷ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation document](#), 7 August 2025

⁸ British College of Aesthetic Medicine, [Muscle relaxing injections](#), accessed 1 September 2025

⁹ Cleveland Clinic, [Body contouring](#), accessed 9 September 2025

¹⁰ British College of Aesthetic Medicine, [Chemical peels](#), accessed 1 September 2025

¹¹ DHSC Consultation outcome, [The licensing of non-surgical cosmetic procedures in England: consultation document](#), 7 August 2025

	birthmarks, broken facial veins and rosacea. It can also be used for hair removal.
Laser hair removal	In this procedure, a laser emits a light that is absorbed by the pigment (melanin) in the hair. ¹² The light energy is converted to heat, which damages the hair follicles that produce hairs. This damage inhibits or delays hair growth.
Light-emitting diode (LED) treatments	LED light therapy is administered using masks, lamps or handheld devices. The process is said to stimulate the production of collagen and elastin, which improve firmness, reduce fine lines and promote wound healing. It can be used on various skin conditions and concerns, such as acne, fine lines and psoriasis as light works directly on photoreceptors in the skin. It may also be used for facial 'rejuvenation'. ¹³
Microdermabrasion	In this process, an abrasive instrument is used to gently sand the skin and remove its thicker, uneven outer layer. It helps to correct skin discolouration, reduce fine lines and wrinkles and reduce acne scars. ¹⁴
Microneedling, also known as collagen induction therapy	Microneedling uses fine needles to make miniscule micro-injuries to the skin. This triggers the body's wound healing process and collagen production, stimulating healthy surrounding tissue to repair the damage and rejuvenate the skin. Techniques include a manual derma-roller or an electronic microneedling device. No-needle alternatives stimulate exfoliation of the skin without puncturing the skin. ¹⁵
Permanent make-up	Permanent make-up is also referred to as cosmetic tattooing or micropigmentation. Specialised pigments are injected into the upper layer of skin to enhance or mimic the appearance of conventional make-up. It is commonly used on the eyebrows, lips and around the eyes.
Semi-permanent fillers	Dermal and lip fillers both contain hyaluronic acid, a naturally occurring substance in the body that helps to hydrate and plump the skin. ¹⁶ When injected into the lips, fillers are used to increase volume and define lip borders. When injected into the face, fillers are used to increase volume and enhance or reshape facial features without surgery. ¹⁷ They can also help to smooth wrinkles.

¹² Mayo Clinic, [Laser hair removal. Overview](#), accessed 1 September 2025

¹³ DHSC, Consultation outcome, [The licensing of non-surgical cosmetic procedures in England: consultation document](#), 7 August 2025

¹⁴ American Society of Plastic Surgeons, [What is microdermabrasion](#), accessed 1 September 2025

¹⁵ DHSC, Consultation outcome, [The licensing of non-surgical cosmetic procedures in England: consultation document](#), 7 August 2025

¹⁶ British College of Aesthetic Medicine, [Dermal fillers](#), accessed 1 September 2025

¹⁷ British College of Aesthetic Medicine, [Dermal fillers](#), accessed 1 September 2025

1.2

Availability on the NHS

Non-surgical cosmetic procedures are not available within the NHS. Some products commonly used in cosmetic procedures may be recommended for medical purposes within the NHS. An example is the use of botulinum toxin ('Botox') for the treatment of cerebral palsy and other neurological conditions that involve muscle stiffness and pain.¹⁸

¹⁸ Great Ormond Street Hospital for Children, [Botulinum toxin injections](#), accessed 8 September 2025

2

Concerns about safety and a lack of regulation

Overview

There have been longstanding concerns about a lack of regulation in the cosmetic procedure sector.

In 2013, the Keogh Review (see section 2.1 below) described the sector as being “highly fragmented” and concluded that attempts at self-regulation within the industry had “largely failed” because of the diversity of the sector and a lack of mandatory requirements.¹⁹ The report also highlighted challenges relating to enforcement and limited opportunities of redress unless negligence could be demonstrated.

The review also described legislative arrangements for the non-surgical cosmetic procedures sector as having been developed in a “piecemeal fashion, addressing certain aspects of the sector but not taking a systematic approach”.²⁰

The [2017 Nuffield report](#) (see section 2.2 below) considered that a number of reports within the last decade had identified “significant gaps and flaws” in the regulatory framework for cosmetic procedures, suggesting that the Keogh Review had “remedied some, but not all, of these”.²¹

[Karin Smyth, Minister of State for Health, said the](#) “cosmetic industry has been plagued by a Wild West of dodgy practitioners and procedures”.²²

Paul Harris, former president of the British Association of Aesthetic Plastic Surgeons (BAAPS), [expressed a number of concerns about regulatory arrangements](#), citing a potential for “profit to be placed before patient care”, the ability of young people to access procedures and the dangers of emergency complications being managed outside of a medical environment.²³ He called for products, practitioners, procedures and premises to undergo urgent, further regulation to ensure patients’ physical and psychological well-being.

¹⁹ DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

²⁰ DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

²¹ Nuffield Council on Bioethics, [Cosmetic procedures: ethical issues](#), 22 June 2017

²² DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

²³ [Cosmetic surgery stats: number of surgeries remains stable amid calls for greater regulation of quick fix solutions](#), BAAPS, 20 May 2019

Save Face, a voluntary accreditation body for cosmetic practitioners (see section 4), has highlighted concerns about the safety and quality of procedures being carried out by practitioners that are not subject to any regulatory controls.²⁴

2.1

The Keogh Review (2013)

In 2010, concerns emerged about the safety and quality of Poly Implant Prothèse (PIP) breast implants. PIP implants are used to increase or enhance breast size or shape, as part of a surgical procedure. An NHS webpage [on PIP breast implants](#) provides background.²⁵

In 2012, an expert group led by Sir Bruce Keogh reviewed evidence concerning the potential health risks associated the implants. Sir Bruce was a cardiac surgeon at the time of appointment and is currently medical director for NHS England. The group published their findings in a [2012 report](#).²⁶ The PIP breast implant scandal raised concerns about the safety and regulation of surgical and non-surgical cosmetic procedures.

In light of these concerns, the then Secretary of State for Health, Andrew Lansley, asked Sir Bruce Keogh to form an expert group to “look at how the safety of patients considering cosmetic interventions can be better ensured in the future”.²⁷

The expert group was to review the arrangements for ensuring the quality and safety of cosmetic interventions that pose a risk to health.

The [Review of the Regulation of Cosmetic Interventions](#) was published in April 2013 and focussed on three main areas:

- high quality care with safe products and responsible providers,
- an informed public to ensure advice is accurate and the vulnerable are protected, and
- accessible redress in the case of things going wrong.²⁸

The review highlighted concerns in several areas, such as the lack of protection for people undergoing non-surgical interventions. It also set out

²⁴ Save Face, [The House – Parliament’s Magazine – Botched and Botoxed: The ‘wild west’ of cosmetic procedures](#), accessed 18 June 2024

²⁵ NHS, [PIP breast implants](#), accessed 1 September 2025

²⁶ [Poly Implant Prothèse \(PIP\) breast implants: final report](#), DHSC, 18 June 2012

²⁷ HC Deb, [Oral questions, Breast implants](#), 11 Jan 2012, col 183

²⁸ DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

concerns about a lack of accredited training courses for practitioners and a lack of restrictions about who could establish training courses.

The review made several recommendations intended to ensure high quality care in the cosmetic procedure sector, sufficient education, training and regulation of practitioners, and redress for members of the public when things go wrong.

Key recommendations included the introduction of a national licensing scheme for practitioners carrying out non-surgical procedures, the licensing of premises used for this purpose, and a requirement for these practitioners to undertake mandatory training and gain accreditation.

In February 2014, the government published its [response](#) to the review's recommendations, saying that they “fully accept[ed] the principles of the Keogh Review and the overwhelming majority of its recommendations for protecting people who choose cosmetic procedures”.²⁹

2.2

The Nuffield Council on Bioethics report (2017)

The Nuffield Council on Bioethics is an independent research centre focusing on biomedicine and health policy.

Prompted by the increased uptake in cosmetic procedures, the council set out to consider the ethical, social and regulatory challenges posed by the UK cosmetic procedures industry.

The council set out to consider several issues related to the increased uptake of cosmetic procedures; a lack of proper regulation, the commercial nature of cosmetic procedures, and the resultant ethical implications for practitioners, service users, as well as those responsible for marketing, advertising and facilitating access to these services. The council therefore established an expert Working Party in 2015, to explore these issues, and published its report [Cosmetic procedures: ethical issues](#), in 2017.³⁰

The report made 27 recommendations in total, including some focused on improving regulation around non-surgical procedures and ensuring the adequate training, qualification and regulation of practitioners. It called for tighter controls on the advertising and marketing of cosmetic procedures, improvements in the provision of informed consent and more stringent measures to regulate access to vulnerable groups.

The report also endorsed the Keogh Review in full, expressing support for the implementation of its recommendations.

²⁹ DHSC, [Government response to the review of the regulation of cosmetic interventions](#), 13 February 2014

³⁰ Nuffield Council on Bioethics, [Cosmetic procedures: ethical issues](#), 22 June 2017

In response to a [parliamentary question asking if the government would implement the report's recommendations](#), the then Minister of State at the Department of Health, Philip Dunne, said the report would help to inform the government's thinking about effective registration and regulation of those performing cosmetic procedures.³¹

2.3

Health Education England review (2013 – 2015)

Health Education England (HEE) is responsible for education, training and workforce development in the health sector, and is now part of NHS England.

The then Department of Health commissioned HEE to:

- conduct a review of the qualifications required for non-surgical cosmetic interventions and the qualifications required to be responsible prescribers (phase one)
- make recommendations on accreditation of qualifications and course delivery (phase two).³²

The HEE review provided a draft framework for qualification requirements. It acknowledged that it would take time to implement new qualification requirements, and that it would not expect practitioners to stop carrying out procedures while they worked towards accreditation.

Phase One report (September 2014)

HEE's [Phase One report](#) set out a need for accredited training for every practitioner.³³ The report set out a draft education and training framework for five treatment modalities; botulinum toxin, dermal fillers, chemical peels and skin rejuvenation treatments, laser, Intense Pulsed Light (IPL) and Light Emitting Diode (LED) treatments and hair restoration surgery.

The review also considered qualifications for prescribing, proposals to introduce legislation to regulate dermal fillers and plans to introduce a prescription regime in which responsibility for prescribing and administering dermal fillers would be held by a regulated clinical practitioner.

³¹ [PQ 8395](#), 12 September 2017

³² Health Education England, [Review of qualifications required for delivery of non-surgical cosmetic interventions: Final report, Phase 1](#), September 2014

³³ Health Education England, [Review of qualifications required for delivery of non-surgical cosmetic interventions: Final report, Phase 1](#), September 2014

Part One report (November 2015)

[Part One](#) of the final report summarises HEE's recommended qualification requirements for practitioners delivering cosmetic procedures.³⁴ The qualification requirements were developed to "support improvements in the quality and standards of patient and client safety and protection in the delivery of cosmetic procedures".³⁵ Part One acknowledges concerns from other stakeholders about the regulatory landscape, who caution that "there are currently no restrictions on who may perform cosmetic procedures, no qualification requirements and an absence of accredited training courses...". HEE recommended qualification requirements for the following treatments:

- Hair restoration surgery
- Botulinum toxins
- Dermal fillers
- Lasers, intense pulsed light and light emitting diode
- Chemical peels and skin rejuvenation

Part Two report (November 2015)

[Part Two](#) provides a detailed account of the qualification requirements for the delivery of non-surgical cosmetic interventions and hair restoration.³⁶ HEE made a number of recommendations concerning the qualification standards, including that they be adopted as best practice and accepted as the industry standard and around the accreditation of courses and providers.

Government response

The government acknowledged publication of the report, and said the Joint Council of Cosmetic Practitioners would look to build on HEE's work and aim to improve the standards and safety of the cosmetic industry.³⁷

³⁴ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

³⁵ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

³⁶ Health Education England, [Part Two: Report on implementation of qualification requirements for cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

³⁷ [PQ 56857](#), 14 December 2016

2.4

All-Party Parliamentary Group on Beauty, Aesthetics and Wellbeing report (2021)

In 2021, the All-Party Parliamentary Group (APPG) on Beauty, Aesthetics and Wellbeing published its concluding report on [its inquiry into advanced aesthetic non-surgical cosmetic treatments](#) (PDF).³⁸

It expressed concern about a lagging regulatory landscape for cosmetic procedures, and also about an insufficient response to the recommendations made in the Keogh Review.

The APPG made several recommendations, including that the government set national minimum training standards for practitioners and introduce a national licensing scheme.

2.5

Health and Social Care Committee report (2020)

In July 2022, the Health and Social Care Committee published its report, [The Impact of body image on mental and physical health](#).³⁹

The committee made several recommendations relating to the regulation of cosmetic procedures. It called for the introduction of:

- a national licensing scheme for practitioners,
- minimum standards for the education and training of practitioners, and oversight of training providers by the [Professional Standards Authority](#),
- the establishment of a ‘Non-Surgical Cosmetic Procedures safety taskforce’. This would include representation from several regulatory bodies, including the Medicines and Healthcare Regulatory products Agency (MHRA), Joint Council for Cosmetic Practitioners (JCCP) and other industry body.

³⁸ All-Party Parliamentary Group on Beauty, Aesthetics and Wellbeing, Concluding report: [Inquiry into advanced aesthetic non-surgical cosmetic treatments](#) (PDF), 2021

³⁹ House of Commons Health and Social Care Committee, [The impact of body image on mental and physical health](#), Second Report of Session 2022–23, HC 114 (PDF), 2 August 2022

The government published its [response to the report](#) (PDF) in February 2023.⁴⁰ In it, the government referred to new powers it had, under the [Health and Care Act 2022](#), to establish a new cosmetic licensing scheme for England.

The government highlighted work undertaken by the JCCP to develop a competency framework for high-risk procedures and noted the availability of Ofqual-approved qualifications. The government said it would continue to work with the JCCP and other stakeholders to consider whether further education and training requirements were necessary.⁴¹

The government also highlighted existing relationships between agencies working on cosmetic procedures, including the Department of Health and Social Care and all professional healthcare regulators.

⁴⁰ [Government response to the House of Commons Health and Social Care Committee's second report of session 2022 to 2023 on the impact of body image on mental and physical health](#) (PDF), February 2023

⁴¹ [Government response to the House of Commons Health and Social Care Committee's second report of session 2022 to 2023 on the impact of body image on mental and physical health](#) (PDF), February 2023

3

Government proposals for a new cosmetic licensing scheme for England

The government has set out an intention to use powers made available in the [Health and Care Act 2022](#) to introduce a new licensing scheme for cosmetic procedures.

Between September and October 2023, the Department of Health and Social Care (DHSC) held a [consultation on the design of the new scheme](#).⁴²

The government published its [response to the consultation in August 2025](#), where it set out details of its proposals on how the new scheme would be implemented.⁴³

3.1

The Health and Care Act 2022

Section 180 of the [Health and Care Act 2022](#) enables the UK Government to establish a licensing scheme for non-surgical cosmetic procedures for England and Wales. In the explanatory notes to the bill, the government set out concerns about the existing regulatory framework, in terms of who can perform cosmetic procedures:

The current regulatory framework places few restrictions on who may perform non-surgical cosmetic interventions. There is also a lack of nationally recognised requirements or standards covering the education, training and qualifications required for the administration of these treatments. There is also no assurance that the premises in which non-surgical cosmetic interventions are carried out meet hygiene standards, or that individuals carrying out procedures have appropriate indemnity and insurance arrangements in place.⁴⁴

The act defines a cosmetic procedure as any procedure (other than a surgical or dental procedure) that is carried out for cosmetic purposes that also includes a reference to:

1. the injection of a substance;

⁴² DHSC, [Licensing of non-surgical cosmetic procedures](#), accessed 4 September 2025

⁴³ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁴⁴ Explanatory notes, [Health and Care Act 2022, Chapter 31](#) (PDF), 2022

2. the application of a substance that is capable of penetrating into or through the epidermis;
3. the insertion of needles into the skin;
4. the placing of threads under the skin;
5. the application of light, electricity, cold or heat;

The government said the act's definition of cosmetic procedures was intended to include procedures considered to have the highest potential to cause harm, while also allowing flexibility for new procedures to be covered by regulations in the future.⁴⁵

The government said the aims of the licencing scheme are to:

- introduce nationally recognised requirements and standards covering the education, training and qualifications required for the administration of those treatments covered by the scheme
- provide assurance that the premises in which non-surgical cosmetic interventions are carried out meet hygiene standards
- ensure that individuals carrying out procedures have appropriate indemnity and insurance arrangements in place⁴⁶

3.2

Consultation on a new licensing scheme for England

Section 180(4) of the [Health and Care Act 2022](#) requires the government to hold a consultation before introducing regulations about the establishment of a new licensing scheme.

Between September and October 2023, the Department of Health and Social Care (DHSC) held a [consultation on the design of the new scheme](#).⁴⁷ The government received 11,848 responses to the consultation. 55% of responses were from those sharing their professional views, whilst 41% were sharing their personal views and experiences.

It invited views from the public and professionals on how to regulate treatments such as botulinum toxin ("Botox") and dermal fillers. The consultation asked a range of questions, including which procedures should

⁴⁵ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), updated 7 August 2025

⁴⁶ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), updated 7 August 2025

⁴⁷ DHSC, [Licensing of non-surgical cosmetic procedures](#), accessed 4 September 2025

be included in a new licensing scheme, and who should be permitted to carry procedures out.

The government published its [response to the consultation](#) in August 2025.⁴⁸

The government proposed placing cosmetic procedures in red, amber or green categories according to the level of risk associated with them. Local authorities would create a licensing scheme for practitioners wishing to perform green and amber procedures, whereas red procedures would be restricted to healthcare professionals only and regulated by the Care Quality Commission. The government also set out an intention to restrict procedures from being delivered to under-18s, except with medical oversight.

The detail relating to the consultation's key proposals are set out in the relevant sections of this briefing.

Stakeholder response

Professor David Sines, Executive Chair and Registrar at the Joint Council for Cosmetic Practitioners (JCCP), said the JCCP “warmly welcomes the UK government’s decision to introduce legislation to regulate and licence cosmetic procedures according to the risks they present”.⁴⁹

Ashton Collins, Director at Save Face, said she was:

delighted that the government has recognised the significant and potentially fatal risks posed by highly dangerous procedures like liquid Brazilian butt lifts, and has made it a priority to implement restrictions to protect public safety.

She said Save Face looked forward to working with the government and stakeholders “to build a safer landscape for the public and promote high standards across the industry”.⁵⁰

Millie Kendall OBE, Chief Executive Officer at the British Beauty Council, welcomed the proposals and said the council would work closely with stakeholders to ensure the measures are “implemented in a way that ensures the sustained, and safe, growth of our sector”.⁵¹

⁴⁸ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁴⁹ DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

⁵⁰ DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

⁵¹ DHSC and Karing Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

4

Regulating practitioners and procedures

At present, there are few restrictions on who can perform non-surgical cosmetic procedures in England. Unlike dentistry, which is tightly regulated under the [Dentists Act 1984](#), or certain “special treatments” such as tattooing and acupuncture regulated by local authorities, anyone can currently provide treatments such as botulinum toxin or dermal fillers.

Voluntary registers, such as the Joint Council for Cosmetic Practitioners (JCCP) and Save Face, set standards for training, competence, and insurance, but practitioners are not legally required to join them. Practitioners who are also registered healthcare professionals are also subject to oversight from their professional regulators.

The government has faced numerous and persistent calls to create a mandatory register for practitioners, based on concerns about the need for accredited training, safe practice, and the protection of patients.

In response to ongoing safety concerns, the government [held a consultation in 2023 on a new licensing scheme](#), that will be introduced using powers set out in the [Health and Care Act 2022](#).

In its response to the consultation, the government proposed a new scheme that would categorise procedures, as discussed in section 3.2 above.

The government’s proposed tiered licensing system would categorise non-surgical cosmetic procedures by risk. Low-risk procedures would be performed by any practitioner meeting agreed standards, medium-risk procedures would require licensed practitioners with supervision for non-healthcare professionals, and high-risk procedures would be restricted to qualified healthcare professionals under the regulation of the Care Quality Commission.

4.1

Who is permitted to provide cosmetic procedures?

At present, anyone can perform non-surgical cosmetic procedures in England, as there are no specific restrictions on who is permitted to do so.

In 2017, the Nuffield Council on Bioethics published its report, [Cosmetic procedures: ethical issues](#) (see section 2.2).⁵² The report noted that, besides limitations on access to prescription medicines and on procedures in the mouth, there are no controls on who is permitted to provide non-surgical procedures.

Regulation of cosmetic dentistry

Regulation in the dental sector is more defined than that of the cosmetic procedure industry. Section 37 of [the Dentists Act 1984](#) sets out which procedures fall within the scope of dentistry. The [General Dentistry Council has published guidance](#) clearly identifying tooth whitening as being within the scope of dentistry.⁵³ Section 38 of the act prohibits anybody who is not a dentist or dental professional, from practicing dentistry.

Local authority licensing scheme for special treatments

[The Local Government \(Miscellaneous Provisions\) Act 1982](#) enables local authorities in England to regulate acupuncture, tattooing, semi-permanent skin-colouring, cosmetic piercing and electrolysis providers. Local authorities can set requirements as to the cleanliness of premises, practitioners and equipment.

[The London Local Authorities Act 1991](#) requires premises used for specified treatments in London. These treatments are acupuncture, tattooing, semi-permanent skin-colouring, cosmetic piercing and electrolysis.

Local authorities can set requirements as to the qualifications of the practitioner, safety requirements for the premises and the cleanliness and hygiene of the premises and equipment.

4.2

Voluntary registers for practitioners

There is no requirement for cosmetic practitioners in England to join a professional register. However, they may join a voluntary register if they choose to.

The Joint Council for Cosmetic Practitioners (JCCP)

In 2016, the British College of Aesthetic Medicine and British Association of Cosmetic Nurses announced plans to jointly establish the [Joint Council for](#)

⁵² Nuffield Foundation, [Cosmetic procedures practice and promotion “cause for serious concern”](#), says [Nuffield Council on Bioethics](#), June 2017

⁵³ General Dental Council, [Frequently asked questions](#), accessed 20 August 2024

[Cosmetic Practitioners \(JCCP\)](#) in response to recommendations made by Higher Education England in its second report on qualification requirements. (see section 7.2).⁵⁴

The JCCP maintains a voluntary register of practitioners who provide non-surgical cosmetic procedures. The JCCP works alongside the [Cosmetic Practice Standards Authority \(CPSA\)](#), an expert group of clinical specialists with patient and public representation. The CPSA sets the training and competency standards for the JCCP register.

Practitioners who are registered healthcare professionals can apply to join Part A of the register. Other practitioners can apply to join Part B of the register.⁵⁵

Registrants must make a declaration of competence and good character and agree to practice within the JCCP/CPSA supervision requirements and code of practice.⁵⁶ Registrants must ensure they only practice within their self-declared scope of practice and demonstrate that they hold suitable insurance and indemnity arrangements. Registrants must also provide evidence of having undertaken training that is suitable for their work.

Registrants must adhere to the JCCP/ CPSA treatment standards (see section 4.4), JCCP [code of practice](#) and the [CPSA supervision matrix](#).⁵⁷

The JCCP register has been formally accredited by the Professional Standards Authority.⁵⁸

The JCCP has also hosts a register of approved education and training providers, which enables practitioners to know which courses and programmes meet the required JCCP/CPSA standards. This register has not been accredited by the Professional Standards Authority, and [requirements for joining are also set out on the JCCP website](#).

Save Face

[Save Face](#) maintains a national register of accredited practitioners who provide non-surgical cosmetic treatments. The register is accredited by the Professional Standards Authority.

Save Face only permits registration for registered doctors, dentists, nurses, midwives and prescribing pharmacists. They must hold suitable indemnity insurance.

⁵⁴ Health Education England, [Part Two: Report on implementation of qualification requirements for cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

⁵⁵ JCCP, [Practitioner Register Committee – Definitions for Members of the Public \(PDF\)](#), accessed 2 September 2025

⁵⁶ JCCP, [Practitioner Register Committee – Definitions for Members of the Public \(PDF\)](#), accessed 2 September 2025

⁵⁷ JCCP, [Joining the Education and Training Register](#), accessed 8 September 2025

⁵⁸ JCCP, [Practitioners, How can the JCCP help me as a ‘Practitioner’?](#), accessed 2 September 2025

Save Face has published information about its [standards for accreditation](#).

4.3

Calls to create a mandatory register for cosmetic practitioners

There is no centralised register of non-surgical practitioners in England and practitioners are not required to complete a registration scheme or otherwise. They can, however, join a voluntary register if they wish (see section 4.2).

The government has faced many calls to introduce a mandatory register for cosmetic practitioners.

The Keogh Review (2013) recommended a centrally maintained register for all practitioners, independent of professional or commercial bodies, to ensure patient safety.⁵⁹

In its response to the Keogh Review, the government stated that creating a new regulated profession was not the only way to improve patient safety, noting that many practitioners were already registered healthcare professionals.⁶⁰ It emphasised that clinical oversight of certain non-surgical procedures could raise standards for unregulated practitioners and said legislative options were being explored in collaboration with professional regulators.

Health Education England and its stakeholder network expressed concern that without statutory regulation, accredited qualifications alone would be difficult to enforce or monitor effectively.⁶¹

In April 2021, representatives of the Chartered Institute for Environmental Health, the Royal Society for Public Health, the Institute of Licensing, the JCCP, UK Public Health Network, the Faculty of Public Health and Save Face [wrote to the then Minister for Patient Safety](#) calling for a national licensing scheme for all cosmetic practitioners to ensure competence and public safety.⁶² The government has not published a formal response to the letter.

The All-Party Parliamentary Group on Beauty, Aesthetics and Wellbeing (APPG) reiterated these concerns in 2021, highlighting gaps in regulation and

⁵⁹ Department of Health, [Review of the Regulation of Cosmetic Interventions, Final Report](#), April 2013

⁶⁰ Department of Health, [Government Response to the Review of the Regulation of Cosmetic Interventions, Cm8776](#), February 2014

⁶¹ Health Education England, [Review of qualifications required for delivery of non-surgical cosmetic interventions](#), September 2014

⁶² The Chartered Institute for Environmental Health, the Royal Society for Public Health, the Institute of Licensing, the JCCP, UK Public Health Network, the Faculty of Public Health and Save Face, [Letter to Nadine Dorries re national licensing scheme for all cosmetic treatments](#), 10 Apr 2021

recommending national minimum training standards alongside a licensing scheme.⁶³

In 2022, the Health and Social Care Committee published its report, [The Impact of body image on mental and physical health](#), in which it recommended that the government create a national licensing scheme.⁶⁴

The government published its [response to the report](#) (PDF) in February 2023.⁶⁵ In it, the government referred to new powers it had been granted under the [Health and Care Act 2022](#), to establish a new cosmetic licensing scheme for England.

4.4

Voluntary standards for procedures

The [Joint Council for Cosmetic Practitioners \(JCCP\)](#) maintains a voluntary register of practitioners who provide non-surgical cosmetic procedures. The JCCP works alongside the [Cosmetic Practice Standards Authority \(CPSA\)](#), which sets the training and competency standards for the JCCP register.

The JCCP has [produced treatment standards](#) that define the level of practice expected from its registrants when delivering five procedures; botulinum toxin ('Botox'), dermal fillers, skin rejuvenation, micro needling and peels, lasers and light (LIPLD) and hair restoration.

3 Background on the JCCP / CPSA

Following its review of qualification requirements for non-surgical practitioners, Health Education England (HEE) put forward its recommended education and training framework for five treatment modalities:

- botulinum toxin,
- dermal fillers,
- chemical peels and skin rejuvenation treatments,
- laser, Intense Pulsed Light (IPL) and Light Emitting Diode (LED) treatments, and
- hair restoration surgery.

⁶³ All-Party Parliamentary Group on Beauty, Aesthetics and Wellbeing, Concluding report: [Inquiry into advanced aesthetic non-surgical cosmetic treatments](#) (PDF), 2021

⁶⁴ House of Commons Health and Social Care Committee, [The impact of body image on mental and physical health](#), Second Report of Session 2022–23, HC 114 (PDF), 2 August 2022

⁶⁵ [Government response to the House of Commons Health and Social Care Committee's second report of session 2022 to 2023 on the impact of body image on mental and physical health](#) (PDF), February 2023

The qualification requirements reflect areas that were highlighted in the Keogh Review, such as obtaining informed consent, infection control, information governance, safe premises and reporting adverse incidents. They also address recommendations for training in physiology, anatomy, infection control, anaphylaxis treatment and understanding pre-existing medical conditions.

The qualification requirements correspond with different levels of learning, reflecting the complexity and risk level of different procedures. Each level of learning corresponds to one of the following:

- Level 4 (equivalent to foundation degree year one level),
- Level 5 (foundation degree level),
- Level 6 (graduate or degree level), or
- Level 7 (postgraduate level).

These standards, set by the CPSA, outline the specific training and education requirements practitioners must complete before providing each treatment modality.

For example, the treatment standard for botulinum toxin defers to the education and training standards recommended by Health Education England (HEE).⁶⁶ HEE recommended that practitioners should have obtained a qualification at level 7 (postgraduate level) to administer botulinum toxin treatments.⁶⁷ HEE also recommended that practitioners should only be able to practice with the oversight of an independent prescriber (see box 5 below). HEE made an identical recommendation for the administering of temporary and semi-permanent dermal fillers.

The treatment standards require practitioners to meet standards of patient safety, professionalism, and competence. This includes explaining risks and obtaining informed consent, allowing a cooling-off period, using approved equipment, ensuring safe waste disposal, preparing for medical emergencies, maintaining procedure records, completing continuing professional development (CPD), working under appropriate supervision and providing follow-up care.

4 Independent prescribers

⁶⁶ Health Education England, [Part Two: Report on implementation of qualification requirements for cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), November 2015

⁶⁷ Health Education England, [Part Two: Report on implementation of qualification requirements for cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), November 2015, see pages 11 and 13

Independent prescribers are healthcare professionals (other than doctors) who have completed further clinical training. This training enables them to assess and diagnose patients, and make decisions about their clinical care, including prescribing. Independent prescribers may also be referred to as ‘non-medical prescribers’.

Independent prescribers can prescribe any medicine within the scope of their practice, and subject to the requirements of medicines legislation.

The type of healthcare professionals that are able to become independent prescribers include nurses, pharmacists, physiotherapists and paramedics.

4.5 Professional membership organisations

Doctors and dentists that provide cosmetic procedures are eligible to join the [British College of Aesthetic Medicine \(BCAM\)](#). Similarly, nurses are eligible to join the [British Association of Medical Aesthetic Nurses \(BAMAN\)](#).

4.6 Healthcare professionals

Many healthcare professionals, commonly nurses, doctors, dentists and pharmacists provide non-cosmetic procedures.

In general, registered healthcare professionals must adhere to codes of conduct, or standards of practice, set by their professional regulator such as the General Medical Council or the General Pharmaceutical Council.

This is also likely required to maintain membership with professional membership bodies, for example the Royal College of Nursing or the British Dental Association.

Regulators and professional bodies will generally set out a requirement for registrants and members to:

- carry out their work safely and effectively,
- prioritise the needs of their patients,
- work within the limits of their competence, and
- hold suitable insurance and indemnity arrangements.

Healthcare professionals are expected to meet these requirements even when working outside their normal area of practice.

Professional guidance

In addition to codes of conduct and professional standards, regulators and professional bodies often publish guidance to support healthcare professionals in key areas of practice. The guidance aims to provide a framework to support registrants to meet professional standards, clinical best practice and statutory requirements.

Professional guidance is not typically mandatory however compliance usually demonstrates that the professional is meeting the relevant requirements.

Regulatory bodies may investigate registrants where concerns are raised about the safety or quality of their practice. If the registrant has not followed relevant guidance, they may need to demonstrate how they have otherwise met the relevant regulatory and statutory requirements.

Regulators and professional bodies have published a range of guidance to support registrants where they choose to undertake cosmetic practice, such as the General Medical Council's [guidance for doctors who offer cosmetic interventions](#).⁶⁸

Cooperation between voluntary registers, regulators and professional bodies

The JCCP has negotiated [memorandums of understanding with a range of regulators and professional membership bodies](#).⁶⁹ These outline how the JCCP, regulators and professional bodies will work together if concerns arise about the practice of a healthcare professional that is registered with both organisations.

If Save Face receives a “high risk” complaint about a registrant, it will refer the matter to the registrant’s professional body.⁷⁰ Save Face has explained that it will share information with that professional body, as required, and may provide an account to that regulator in the course of its investigation.⁷¹

Indemnity insurance

Another general condition of registration for professional regulators is that registrants hold indemnity insurance that is relevant to their area of practice.

Insurance can be provided by an employer or arranged independently by the healthcare professional. Insurers will generally require healthcare professionals to work only within the limits of their competence and may require proof of professional accreditation or training to demonstrate this.

⁶⁸ General Medical Council, [Cosmetic interventions](#), accessed 2 September 2025

⁶⁹ JCCP, [About us – JCCP external partnership agreements, policies and procedures](#), 2 September 2025

⁷⁰ Save Face, [Complaint investigation decisions](#), accessed 2 September 2025

⁷¹ Save Face, [Complaint investigation decisions](#), accessed 2 September 2025

4.7

New government proposal for a 3-tier licensing scheme for cosmetic procedures

Between September and October 2023, the Department of Health and Social Care (DHSC) held a [consultation on the design of a new scheme for the licensing of cosmetic procedures](#).⁷²

The government published its [response to the consultation](#) in August 2025.⁷³ The government proposed to categorise procedures, according to their level of complexity and degree of invasiveness.

Green procedures

These would be procedures with the lowest risk of complications, such as micropigmentation (semi-permanent make up), intense pulsed light (IPL) and light emitting diode (LED) therapies. All practitioners would be eligible to carry these procedures out, provided they meet agreed standards. Green procedures would fall within the scope of the new licensing scheme, brought into effect using powers in the Health and Care Act 2022.

Amber procedures

These would be procedures with a medium risk of complications, such as the injection of botulinum toxins and semi-permanent dermal fillers into the face.

Non-healthcare professionals would need to be licensed and be supervised by a named, regulated healthcare professional. That professional would need to have gained an accredited qualification to prescribe, administer and supervise cosmetic procedures.⁷⁴

Regulated healthcare professionals would be able to perform these procedures without supervision, provided they meet the agreed standards. Amber procedures would fall within the scope of the new licensing scheme, brought into effect using powers in the Health and Care Act 2022.

Red procedures

These would include procedures with the highest risk of complications, such as hair restoration surgery, and procedures aimed at augmenting the body with the patient's own fat or dermal fillers.

⁷² DHSC, [Licensing of non-surgical cosmetic procedures](#), accessed 4 September 2025

⁷³ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁷⁴ This is based upon the principles of the clinical oversight model recommended within Professor Sir Bruce Keogh's 2013 [Review of the regulation of cosmetic interventions](#) and Health Education England's, [Qualification requirements for the delivery of non-surgical education and training](#) (2015).

These procedures would be brought under regulation by the Care Quality Commission (CQC) and would be excluded from the licensing scheme. The government has proposed that red procedures would be restricted to qualified and regulated healthcare professionals only.

Consultation response to this proposal

The majority of respondents agreed with the proposal.⁷⁵ Some suggested other ways of managing risk, such as by disregarding a three-tier structure, and focussing instead on training and qualification standards. Some respondents proposed an additional tier, which would restrict certain procedure so that they could only be administered by prescribing healthcare professionals or by non-prescribing healthcare professionals under the supervision of a prescribing healthcare professional.⁷⁶

Some respondents expressed uncertainty about the arrangements for the amber category, because of concerns about what supervision would entail, and the associated practicalities and costs of delivering it.

Some respondents also felt that procedures that are currently regulated under the [Local Government \(Miscellaneous Provisions\) Act 1982](#) (see section 4.1) should be included in this new licensing scheme. These are tattooing, cosmetic piercing, electrolysis, semi-permanent skin colouring, and acupuncture. Some local authorities were concerned that the existing scheme for special procedures is not fit for purpose, and that a separate scheme could result in duplication of oversight and fee requirements for businesses that offer a mix of activities.

Government response

The government noted general support from respondents about the tiered licensing system, but said further work was needed to determine where specific procedures would sit in the tiered system.⁷⁷

The government said it wanted to focus on procedures that are currently subject to little or no regulatory oversight and would not seek to include procedures currently covered by the Local Government (Miscellaneous Provisions) Act 1982 at this time. The government did, however, say that it would continue to work with stakeholders to understand their concerns and prevent bureaucratic duplication.⁷⁸

⁷⁵ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁷⁶ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁷⁷ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁷⁸ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

5

Regulating products and prescribing

Botulinum toxin ('Botox') is currently regulated as a prescription-only medicine. It can only be supplied on prescription, and administration must be carried out by an appropriate practitioner or under their direction. There have been concerns about a lack of face-to-face consultation, between the prescriber and person receiving the procedure. There are also concerns that presently, there is no legal restrictions on who can administer Botox once prescribed, though the prescriber must ensure the practitioner is competent.

Dermal fillers are available for sale, without the need for a prescription. The government has set out an intention to bring dermal fillers into scope for the regulation of medical devices, although this has not yet been implemented.

In its [response to the consultation on the new cosmetic licensing scheme](#), the government highlighted concerns about the lack of face-to-face consultations and the limitations on nurse independent prescribers holding prescription-only medicines, which could delay emergency interventions.⁷⁹ The government proposed that any procedure involving a prescription-only medicine should be overseen by a qualified healthcare professional. It also committed to working with regulators to explore legislative and guidance options to reduce the risks associated with poor practice in the use of prescription-only medicines in cosmetic procedures.

5.1

Current restrictions on prescribing and administering botulinum toxin ('Botox')

In the UK, botulinum toxin is licensed as a muscle relaxant to treat muscle-related conditions. It is not licensed for general cosmetic purposes, so its use in this aspect remains [unlicensed, or off-label](#).

[The Human Medicines Regulations 2012](#) stipulate that botulinum toxin can only be sold or supplied on prescription.⁸⁰ The regulations also prohibit a person from parenterally administering a prescription only medicine to another person unless they are an appropriate practitioner or are acting in accordance with the directions of an appropriate practitioner.⁸¹

⁷⁹ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁸⁰ Schedule 1 of the Human Medicines Regulations 2012 designates all products for parenteral (via injection) administration as prescription only medicines.

⁸¹ [Human Medicines Regulations 2012 \(as amended\)](#), section 214(2)

It is current practice, in the cosmetic procedure sector, for a prescriber to issue a prescription for botulinum toxin, before it is administered by a cosmetic practitioner.⁸²

The General Medical Council has issued guidance stating that prescribers must carry out a physical examination of patients and cannot prescribe the product remotely.⁸³ The General Dental Council states dentists must carry out a “full assessment” of the patient before prescribing botulinum toxin.⁸⁴

Prescribers are responsible for explaining that botulinum toxin is unlicensed for use in cosmetic procedures. Further information on the prescribing of botulinum toxin is available in Health Education England’s Phase One report on qualification requirements for cosmetic interventions.⁸⁵

In practice, this means there are no restrictions on who can administer botulinum toxin, provided it has been appropriately prescribed. However, the prescriber remains clinically responsible and should therefore ensure that the administering practitioner is suitably competent.⁸⁶

The government’s [response to its consultation on the licensing of cosmetic procedures in England](#) said respondents had expressed concern about a lack of face-to-face consultations between prescribers and clients for procedures that require prescription-only medicines.⁸⁷ The government said it would continue to work with regulatory bodies to explore how legislation and guidance could be used to “mitigate the risks which poor practice in these areas pose”.⁸⁸

5.2

Dermal fillers

Dermal fillers that do not make medical claims are not currently regulated as medical devices.⁸⁹ They can be sold or supplied without a prescription, regardless of whether they are classed as medical devices or not. There are no restrictions on who is permitted to administer dermal fillers.

⁸² Health Education England, [Review of qualifications required for delivery of non-surgical cosmetic interventions: Final report, Phase 1](#), September 2014, pages 34-35

⁸³ General Medical Council, [Questions about our cosmetic interventions guidance](#), accessed 3 September 2024

⁸⁴ General Dental Council, [Frequently asked questions](#), accessed 3 September 2024

⁸⁵ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), November 2015, pages 32-33

⁸⁶ Health Education England, [Review of qualifications required for delivery of non-surgical cosmetic interventions: Final report, Phase 1](#), September 2014, page 33

⁸⁷ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁸⁸ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁸⁹ MHRA, [Freedom of Information request \(FOI 22/901\)](#), 17 January 2024

Between September and November 2021, the Medicines and Healthcare products Regulatory Agency (MHRA) [held a consultation on amending the scope of UK medical devices regulations](#).⁹⁰ The government proposed broadening UK medical devices regulations, so that they would include devices without a medical purpose, but with a similar risk profile to medical devices. The government included dermal fillers in the list of products that could fall under this expanded scope.⁹¹

The government [published its response to the consultation](#) in June 2022 (PDF).⁹² The government said it intended to proceed with this proposal. At the time of writing, it had not been implemented. In July 2025, [the government said it intended to consult further](#) on aspects of this policy.⁹³

In the consultation response, the government also responded to proposals, set out by respondents, that dermal fillers should only be available on prescription. The government explained that dermal fillers generally cannot be covered by the existing framework for prescription-only medicines because they are medical devices rather than medicinal products.⁹⁴

5.3 New government proposal on restricting access to prescription-only medicines

In its [consultation response on the design of the new cosmetic licensing scheme for England](#), the government noted concern, raised by respondents, about the lack of face-to-face consultations between prescribers and clients for procedures that require prescription-only medicines (POMs).⁹⁵

Respondents also noted that current legislation in England prevented nurse independent prescribers from holding stock of prescription medicines, which could delay their use in emergencies or complications. Examples include the use of adrenaline in the event of anaphylaxis, or hyaluronidase to dissolve dermal filler and prevent the blockage of blood vessels.

The government proposed that any procedure requiring a prescription-only medicine would need to be overseen by a qualified and regulated healthcare professional. The consultation referred to examples such as the use of

⁹⁰ MHRA, [Consultation on the future regulation of medical devices in the United Kingdom](#), last updated 26 June 2022

⁹¹ MHRA, Consultation outcome, Chapter 1: Scope of the regulations, updated 26 June 2022, see section 2.3, paragraph d

⁹² MHRA, [Government response to consultation on the future regulation of medical devices in the United Kingdom](#) (PDF), 26 June 2022

⁹³ [PQ64311](#), 7 July 2025

⁹⁴ [Government response to the House of Commons Health and Social Care Committee's second report of session 2022 to 2023 on the impact of body image on mental and physical health](#) (PDF), February 2023

⁹⁵ DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

medicated injections to reduce hay-fever symptoms, or the topical use of hydroxyquinone to treat pigmentation problems or skin whitening.

The government said it would continue to explore, with relevant regulatory bodies, how legislation and guidance could be used to mitigate the risks that poor practice in these areas pose.⁹⁶

⁹⁶ DHSC and Karin Smyth MP, [Crackdown on unsafe cosmetic procedures to protect the public](#), 6 August 2025

6 Age restriction on cosmetic procedures

In its [response to the consultation on the new cosmetic licensing scheme](#), the government set out an intention to introduce age restrictions on cosmetic procedures.⁹⁷ The government said this would bring restrictions in line with those of other procedures, such as botulinum toxin.

6.1 Current restrictions on botulinum toxin and dermal fillers to under-18s

[The Botulinum Toxin and Cosmetic Fillers \(Children\) Act 2021](#) made it an offence to administer botulinum toxin ('Botox') or dermal fillers to under-18s in England.⁹⁸ Children under 18 can still access these procedures from qualified healthcare professionals where there is a medical need.

The government said the purpose of the bill was to safeguard children from the potential health risks of botulinum toxin and dermal fillers.⁹⁹

Further information about the bill is available in [a Commons Library bill briefing](#).¹⁰⁰

6.2 New government proposal to further restrict procedures for under-18s

In its [consultation response on the design of the new cosmetic licensing scheme for England](#), the government proposed that the new licensing scheme would prohibit practitioners from performing procedures on people under 18, unless approved by a registered doctor and carried out by a specified healthcare professional.¹⁰¹

⁹⁷ DHSC, [The licensing of non-surgical cosmetic procedures in England: consultation response](#), 7 August 2025

⁹⁸ The Act was initially put forward by the DHSC as a '[handout Bill](#)', before being sponsored by Laura Trott MP and Baroness Wyld.

⁹⁹ DHSC, [Explanatory notes, Botulinum toxin and cosmetic fillers \(children\) bill](#) (PDF), 2020

¹⁰⁰ House of Commons Library, [Botulinum Toxin and Cosmetic Fillers \(Children\) Bill 2019-21](#), 10 March 2021

¹⁰¹ DHSC, [Licensing of non-surgical cosmetic procedures](#), accessed 4 September 2025

The government said this would bring cosmetic procedures in line with restrictions on botulinum toxin ('Botox') injections, dermal fillers, tattoos, teeth whitening and sunbed use.

The government's consultation response set out that a majority of respondents were in favour of this proposal. Some respondents proposed a higher age restriction of 21 years or higher.

Some respondents agreed with allowing under 18s to access procedures with the involvement of a doctor or medical professional, with consideration to certain kinds of procedures. They emphasised the importance of treatments for conditions like acne and scarring, excessive body hair and skin lesions, which may be sought to alleviate the psychological and emotional distress that such conditions may cause young people.

However, some expressed concern that this arrangement could prevent young people from accessing treatment in a timely manner. Others questioned if it was a proportionate arrangement given other healthcare professionals' competency and scopes of practice, and the intention for licensing to create consistent standards for all practitioners regardless of their professional background.

The government said the consultation had strengthened its intention to mandate age restrictions for those undergoing non-surgical cosmetic procedures.

7

Accreditation for training providers

The Keogh Review (2013) raised concerns about the lack of accredited training for cosmetic practitioners and recommended that Health Education England (HEE) develop recognised qualifications and set accreditation requirements.¹⁰²

[Health Education England's \(HEE\) 2015 report](#) summarised its recommendations for qualification requirements in the cosmetic procedure sector.¹⁰³ These included recommendations on entry requirements, knowledge and skills, law and ethics, health and safety, and practical assessment. Training providers can choose to adopt HEE's recommendations, but this is not mandatory.

Training providers can choose to adopt other guidance and standards on education material, including those set out by Ofqual and National Occupational Standards.

7.1

Concerns about a lack of accreditation for training providers

The 2013 Keogh Review set out concerns about the absence of accredited training courses for practitioners.¹⁰⁴ It noted that “anyone can set up a training course purporting to offer a qualification”, and the review committee expressed concern that self-accredited training organisations had been established.¹⁰⁵

The review recommended that Health Education England's (HEE) mandate should include the development of accredited qualifications for practitioners, and that HEE should determine accreditation requirements for various professional groups. The review said this work should be completed in 2013.

In its response to the Keogh Review, the government said that HEE would work with regulators, Royal Colleges and other stakeholders to review

¹⁰² DHSC, [Review of the Regulation of Cosmetic Interventions](#), April 2013

¹⁰³ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

¹⁰⁴ Department of Health, [Review of the Regulation of Cosmetic Interventions, Final Report](#), April 2013

¹⁰⁵ Department of Health, [Review of the Regulation of Cosmetic Interventions, Final Report](#), April 2013, para 3.10

training requirements for practitioners.¹⁰⁶ The government said that HEE could, as part of this review, make recommendations on who might be a suitable body to accredit qualifications for cosmetic procedures.

In its 2015 report, HEE recommended that:

all organisations wishing to develop and provide qualifications which meet HEE's requirements and which have not been approved or accredited by a professional regulatory body or Royal College should be regulated by OFQUAL or have their own degree awarding powers or should work in partnership with these organisations to obtain appropriate course accreditation.¹⁰⁷

Ofqual has published [guidance for training providers who choose to have their qualifications regulated by Ofqual](#).¹⁰⁸

7.2

HEE's recommended framework for qualifications

[Health Education England's \(HEE\) 2015 report](#) summarised its recommendations for qualification requirements in the cosmetic procedure sector.¹⁰⁹ The recommendations cover five treatment modalities, four of which cover non-surgical procedures including botulinum toxins ('Botox') and dermal fillers.

In section two of the report, HEE made recommendations as to entry requirements for students/ trainees. For example, the minimum educational qualifications that they should have obtained, as well as ensuring they have the required values and behaviour required of those delivering care to members of the public.

HEE set out indicative content for training providers across four areas;

- generic knowledge and skills,
- speciality specific knowledge and skills,
- law, policy and ethics, and
- facilities, premises, and health and safety.

¹⁰⁶ DHSC, [Government response to the review of the regulation of cosmetic interventions](#), 13 February 2014

¹⁰⁷ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015, page 20

¹⁰⁸ Ofqual, [Apply to have your qualifications regulated](#), last updated 2 September 2025

¹⁰⁹ Health Education England, [Part One: Qualification requirements for delivery of cosmetic procedures: Non-surgical cosmetic interventions and hair restoration surgery](#), Nov 2015

The report made further recommendations regarding assessment and practical training. Training providers can choose to meet the requirements set out in HEE's framework but are not mandated to do so.

7.3 Government comment on qualification requirements

In July 2022, the Health and Social Care Committee published its report, [The Impact of body image on mental and physical health](#).¹¹⁰ The committee recommended, among other things, that the government set minimum standards for the education and training of practitioners, and require that providers be overseen by the Professional Standards Authority.

The government published its [response to the report](#) (PDF) in February 2023, and provided the following response to this recommendation:

The government agrees that those who offer non-surgical cosmetic procedures to the public should be suitably trained and qualified.

We recognise there is a need for nationally recognised standards covering the education, training and qualifications required for the administration of non-surgical cosmetic procedures.

The Joint Council of Cosmetic Practitioners (JCCP) has already developed a competency framework covering high-risk non-surgical cosmetic procedures and there are a limited number of bodies currently able to offer training courses on non-surgical cosmetic procedures. This includes universities, colleges and private training companies.

There are also a range of Ofqual-approved qualifications that are delivered by recognised Ofqual awarding bodies.

We will work with JCCP and other relevant stakeholders to consider whether further education and training requirements are necessary.¹¹¹

7.4 National occupational standards

[National occupational standards \(NOS\)](#) are developed by standard setting organisations, in consultation with employers and other stakeholders across each of the UK nations.¹¹² The standards are not intended as vocational

¹¹⁰ House of Commons Health and Social Care Committee, [The impact of body image on mental and physical health](#), Second Report of Session 2022–23, HC 114 (PDF), 2 August 2022

¹¹¹ [Government response to the House of Commons Health and Social Care Committee's second report of session 2022 to 2023 on the impact of body image on mental and physical health](#) (PDF), February 2023

¹¹² National Occupational Standards, [About NOS](#), accessed 4 September 2025

curriculum but are rather intended to influence the content and development of qualifications and training courses.

Examples of standards relevant to cosmetic practice include; [SKAB38 Provide cosmetic skin needling treatments](#) and [SKABBR2 Provide facial skincare treatments](#).

7.5

JCCP and BAAPS partnership on training standards

In September 2025, the Joint Council for Cosmetic Practitioners (JCCP) and British Association of Aesthetic Plastic Surgeons (BAAPS) set out plans to collaborate on the codesign and improvement of practice standards and guidance for invasive non-surgical procedures.¹¹³

¹¹³ Aesthetics Journal, [JCCP and BAAPS unveil new partnership](#), 1 September 2025

8

Other regulatory matters

Practitioners and businesses providing cosmetic procedures are subject to health and safety law.

Local authorities have powers to close premises or restrict activities under public health law, if they consider there is a risk to public health.

The Care Quality Commission (CQC) regulates a range of health services in England. Whilst non-surgical procedures fall outside of its remit, the CQC may consider complaints about non-surgical procedures if they are provided by a CQC-registered provider, and if the complaint suggests there may also be concerns about the provider's CQC-regulated activities.

8.1

Health and safety requirements

Individuals delivering cosmetic procedures are subject to broad health and safety legislation that is applicable to most work activities. Employers and self-employed practitioners must manage the risk of harm, to anybody that is affected by their work activity.

The Health and Safety Executive (HSE) is the independent regulator for workplace health and safety in Great Britain. The HSE website includes [guidance on health and safety legislation](#).

Depending on the nature of the cosmetic procedure, the following pieces of legislation may be relevant:

- The [Health and Safety at Work etc Act 1974 \(HSWA\)](#) requires employers and the self-employed to undertake their work in such a way as to ensure, so far as is reasonably practicable, that employees and non-employees who might be affected by the work are not exposed to risks to their health or safety.
- [The Management of Health and Safety at Work Regulations 1999](#) (MHSWR) requires employers and the self-employed to carry out a risk assessment. This must identify potential risks, the likelihood that a person might be harmed, and set out measures to eliminate or control the hazard.
- [The Control of Substances Hazardous to Health Regulations 2002 \(COSHH\)](#) requires employers and the self-employed to adequately control exposure to materials in the workplace that cause ill health. These can include dust, gases or fumes that are breathed in, or liquids,

gels and powders that come into contact with your eyes or skin. The HSE has published [guidance on COSHH for beauticians](#).¹¹⁴

- [The Personal Protective Equipment at Work Regulations 1992](#) require employers to provide appropriate personal protective equipment (PPE) for employees, where risks cannot be adequately controlled by other means.
- [The Provision and Use of Work Equipment Regulations 1998](#) place duties on people and companies that own, operate or control work equipment. The regulations require that equipment provided at work is suitable for the intended use, is safe, maintained well and inspected to determine its condition, and is only used by people who have received adequate training.

Enforcing health and safety law

The Health and Safety Executive (HSE) is the regulator for work-related health and safety in Great Britain.

Enforcement for health and safety law is shared between the HSE, local authorities and industry-specific regulators. HSE guidance sets out [enforcement responsibilities for beauty treatments](#).¹¹⁵

Local authorities are responsible where treatments are provided in a salon. They are also responsible where treatments are provided from a domestic premises which has a dedicated separate access/ egress from the main premises i.e. a doorway directly to the outside from the treatment room.

HSE is responsible where treatments are provided in domestic settings. This includes those delivered either in clients' homes by mobile practitioners or in a practitioner's own home where the treatment area is accessed through the main domestic entrance.

Reporting health and safety concerns

The HSE website explains how [members of the public can report concerns to them](#).¹¹⁶ The HSE has also published its [enforcement policy](#), which explains what measures they might take in response to concerns.¹¹⁷ The policy explains that the HSE uses its discretion when deciding whether to investigate incidents. It takes into account the scale of potential or actual harm and the seriousness of any potential breach of the law. The HSE could take a range of actions in response to its investigation, including serving improvement and prohibition notices, issue cautions or prosecute.

¹¹⁴ HSE, [COSHH and beauticians – key messages](#), accessed 8 September 2025

¹¹⁵ HSE, [Appendix: Health and Safety \(Enforcing Authority\) Regulations 1998: A-Z guide to allocation](#), accessed 5 September 2025

¹¹⁶ HSE, [Tell us about a health and safety issue](#), accessed 5 September 2025

¹¹⁷ HSE, [Enforcement Policy Statement \(EPS\)](#), accessed 5 September 2025

Local authority websites will usually include information about how residents can report health and safety concerns, and they may also publish information about their enforcement process.

8.2 Local authority powers under public health law

[The Public Health \(Control of Disease\) Act 1984](#) grants powers to local authorities to apply to a magistrate's court to close premises or prohibit or restrict activities that pose a public health risk. These powers are intended to manage a wide range of public health risks, including the control of communicable diseases and other hazards.

In principle, these powers could be used in relation to premises offering cosmetic procedures if the activities carried out there were judged to pose a public health risk. For example, local authorities could seek court orders to restrict or close premises where unsafe hygiene practices or other factors posed a risk to the health of patients or the wider public.

8.3 CQC-registered premises

[The Care Quality Commission \(CQC\)](#) is the independent regulator of health and adult social care services in England. It is responsible for regulating clinics that provide surgical procedures.

In general, the CQC does not handle complaints about non-surgical cosmetic procedures, as these fall outside its regulatory scope. However, if a non-surgical procedure is carried out by a CQC-registered provider and raises concerns that could also affect the provider's regulated (surgical) activities, the CQC may choose to review the complaint.

While the CQC welcomes information from individuals to help build a broader understanding of how a service is performing, it does not have the authority to investigate or resolve individual complaints. The CQC uses such information to inform its regulatory activities but cannot take forward formal complaints on behalf of individuals.

9

Guidance for members of the public

The NHS has published advice, [Before you have a cosmetic procedure](#), that advises on considerations people should make before proceeding. It advises on how to choose a qualified practitioner, understanding the cost of immediate and future procedures as needed and taking care when choosing providers recommended by influencers on social media.

Other guidance published by the NHS, [Choosing who will do your cosmetic procedure](#), explains what steps people should take to ensure that they are choosing a safe practitioner.

Save Face has published a [Be Safe Checklist](#) for the public to help them make an informed choice about their practitioner, and [information on expressing concerns about practitioners](#).

9.1

Health concerns after undertaking a procedure

People who are concerned about their health or wellbeing following a cosmetic procedure should seek advice from a qualified healthcare professional.

The NHS will treat people experiencing complications or side effects from cosmetic procedures. However, the aim of this treatment will be to ensure a patient's health and safety. The NHS will not generally seek to restore or improve cosmetic appearance.

10

Redress if things go wrong

This section provides an overview of routes that members of the public could consider if they are concerned or unsatisfied with a cosmetic procedure they have undertaken. This information should not be interpreted as advice, and members of the public should seek advice from a relevant professional.

10.1

Internal complaints procedure

If a member of the public is unhappy with any aspect of a cosmetic procedure, they may wish to express their concerns to the practitioner or the practitioner's employer.

Self-employed practitioners and employers may provide a complaints procedure, setting out how members of the public can raise concerns, how they will respond to those concerns, and potential routes of redress if the member of the public remains dissatisfied following the initial stages of the complaints process.

10.2

Raising concerns with voluntary registers

Members of the public can report concerns about practitioners to the voluntary [Joint Council for Cosmetic Practitioners \(JCCP\)](#) register, provided the practitioner is a registrant.

The JCCP considers a wide range of complaints, including those relating to safety concerns.¹¹⁸ The JCCP has published its [complaints procedure](#) (PDF).¹¹⁹

Similarly, members of the public can [report concerns to the voluntary Save Face register](#), if the practitioner is a registrant.¹²⁰

¹¹⁸ JCCP, [Making a complaint or raising an issue of complaint, 2023/04 complaints policy v 1.2 \(PDF\)](#), April 2023

¹¹⁹ JCCP, [Making a complaint or raising an issue of complaint, 2023/04 complaints policy v 1.2 \(PDF\)](#), April 2023

¹²⁰ Save Face, [Raise a concern](#), accessed 5 September 2025

10.3

Voluntary adjudication schemes

The Independent Sector Complaints Adjudication Service (ISCAS)

The [Independent Sector Complaints Adjudication Service \(ISCAS\)](#) provides independent adjudication on complaints about ISCAS subscribers. ISCAS is a voluntary subscriber scheme for many independent healthcare providers. Members of the public [can complain to ISCAS](#) if they are unhappy with a treatment provided by a practitioner working in an ISCAS registered business.

ISCAS has set out details of its four-stage complaints process, including that ISCAS can ask subscribers to take measures to remedy a complaint.¹²¹ This could include offering an apology or a goodwill payment. ISCAS has clarified its remit with regards to compensation and refunds:

compensation is not achievable under the ISCAS Code: the term ‘compensation’ implies that there is a duty on the provider to compensate you for something that went wrong, and to prove that you need to take the matter to the courts. Refunds are also outside the scope of the Code and are a matter for the courts.¹²²

The Cosmetic Redress Scheme (CRS)

[The Cosmetic Redress Scheme \(CRS\)](#) is a consumer redress scheme that cosmetic practitioners can voluntarily join. The CRS is able to resolve complaints, submitted by members of the public, with regards to the consumer aspects of cosmetic services.

The CRS can consider complaints regarding consumer law and poor or unsatisfactory service. The CRS does not consider complaints in relation to dissatisfaction with the results of procedures, damage or harm suffered and medical or criminal negligence.

10.4

Reporting healthcare professionals to professional regulators

If a practitioner is a regulated health and care professional it is possible for members of the public to report concerns about the practitioner’s cosmetic practice, to the regulator.

¹²¹ Independent Sector Complaints Adjudication Service, [What we can do](#), accessed 5 September 2025

¹²² Independent Sector Complaints Adjudication Service, [What we can do](#), accessed 5 September 2025

Typically, the regulator will consider the concerns and decide whether they merit further investigation.

Professional regulators are unlikely to consider complaints about customer service, fees and payments, and those relating to simple dissatisfaction with the cosmetic result. Regulators may consider these concerns where they indicate that the registrant has breached their professional or ethical standards, expected clinical practice, or where the client has been subject to potential or actual harm.

If a complaint is investigated and upheld, regulators can take a range of measures, including issuing a warning, temporarily suspending or fully removing the registrant from the register, or in more serious cases, referring the matter to the police or the Crown Prosecution Service.

Health and care regulators in England include:

- [The General Medical Council \(GMC\)](#), which regulates doctors
- [The Nursing and Midwifery Council \(NMC\)](#), which regulates nurses and midwives
- [The General Dental Council \(GDC\)](#), which regulates dentists and other dental professionals including dental nurses and dental hygienists
- [The General Pharmaceutical Council \(GPhC\)](#), which regulates pharmacist and pharmacy technicians
- [The Health and Care Professions Council \(HCPC\)](#), which regulates over 15 professions including physiotherapists and paramedics

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